

MedAI Clinical Summary

Patient Interview & SOAP Note Documentation

Date: January 11, 2026 at 08:41 AM

■ SUBJECTIVE

Patient reports involvement in a motor vehicle collision approximately 24 hours prior to presentation. Patient was the driver in a rear-end collision, wearing a seatbelt. Denies airbag deployment. Denies loss of consciousness at the scene or any subsequent memory gaps. Primary complaint is new onset neck pain, rated 6/10 at its worst, described as a dull ache with intermittent sharp pains, exacerbated by movement. Reports associated stiffness and muscle spasms in the cervical paraspinal muscles. Also reports concurrent low back pain, 5/10, aching in quality, radiating slightly into the buttocks bilaterally, also worse with movement. Denies any numbness, tingling, or weakness in the extremities. Denies any chest pain, abdominal pain, or other focal injuries. No immediate medical attention was sought after the accident.

■ OBJECTIVE

- General: Alert and oriented, appears mildly uncomfortable, guarding neck movements.
- Head: Normocephalic, atraumatic. Scalp non-tender to palpation.
- Neck: Limited range of motion in flexion, extension, lateral bending, and rotation due to pain and spasm. Tenderness to palpation over cervical paraspinal muscles bilaterally. No midline cervical spine tenderness.
- Back: Limited lumbar range of motion in flexion and extension. Tenderness to palpation over lumbar paraspinal musculature. Negative straight leg raise bilaterally.
- Neurological: Cranial nerves II-XII intact. Motor strength 5/5 symmetrical in all four extremities. Sensation intact to light touch throughout. Deep tendon reflexes 2+ and symmetric.
- Vital signs: Unavailable.

■ ASSESSMENT

- Primary diagnosis:

- Acute cervical strain, post-motor vehicle collision.
- Acute lumbar strain, post-motor vehicle collision.
- Differential diagnoses:
- Concussion.
- Acute thoracic strain.
- Cervical or lumbar radiculopathy.

■ PLAN

- Tests:
- X-rays of cervical spine (AP, lateral, odontoid, oblique views) to rule out fracture or dislocation.
- X-rays of lumbar spine (AP, lateral, oblique views) to rule out fracture or significant malalignment.
- Medications:
- Ibuprofen 600 mg by mouth every 8 hours as needed for pain and inflammation, with food.
- Cyclobenzaprine 5 mg by mouth at bedtime as needed for muscle spasms.
- Patient advice:
- Apply ice packs to affected areas for 20 minutes, several times daily, for the first 48-72 hours, then transition to heat as desired.
- Encourage gentle range of motion exercises as tolerated, avoiding activities that worsen pain.
- Avoid heavy lifting and strenuous physical activity.
- Return for evaluation immediately if experiencing worsening pain, new numbness, tingling, weakness, vision changes, severe headache, or bowel/bladder dysfunction.
- Follow-up instructions:
- Re-evaluate in 1 week to assess symptom progression and review imaging results. Consider referral to physical therapy if symptoms persist.

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